



Other Applications of IFS Therapy

COMMON CONCERNS ABOUT PRACTICING IFS

- 1) I don't have enough experience or training as an IFS therapist.
 - a. Inexperienced IFS therapists can talk to protectors safely but should not attempt to work with exiles.
- 2) I don't feel confident about managing the difficulties that will arise.
 - a. Confidence comes with experience. Therapists who have little experience or training in IFS can still interview protectors safely.
 - b. We also recommend formal training and supervision from an experienced IFS therapist as well as direct experience in IFS therapy.

COMMON QUESTIONS ABOUT PRACTICING IFS

- 1) Can I integrate IFS with my previous training?
 - a. As long as you can be curious, validating, compassionate and polite with the client's parts (that is, be Self-led), feel free to try integrating the skills you know and trust.
 - b. Be aware that IFS does differ from more traditional approaches to trauma therapy in some important ways (Anderson & Sweezy, 2016).
 - c. We recommend doing therapy with an overall IFS framework and integrate other modalities in as you see fit. For example, do a piece of CBT, EMDR or body work with a specific protective part, if it is willing.
- 2) What should I do if an exile is unintentionally triggered?
 - a. Apologize to the client's protectors, let them know this was not your goal and validate their need to do their jobs.

Many seasoned, talented IFS trained therapists are adapting IFS to different therapy populations and therapy modalities in the United States and elsewhere. The list below is not all-inclusive. For more publications and information, please go to The Center for Self-leadership website: <http://www.selfleadership.org>.

THERAPY MODALITIES

Couples

In her adaptation of IFS to couple therapy (Intimacy from the Inside Out or IFIO) Toni Herbine-Blank emphasizes the importance of good communication for intimate relationships. By teaching partners to speak for parts and from the Self, IFIO helps them be with their differences, forego internal as well as external shaming and stay with hard conversations until each feels fully heard, understood and able to live with differences (Herbine-Blank, 2013; Herbine-Blank, Kerpelman & Sweezy, 2016).

Toni Herbine-Blank, MSN: Senior IFS trainer + Training Developer and Director of Intimacy from the Inside Out, www.toniherbineblank.com

Stepfamilies

In her work with stepfamilies, Patricia Papernow uses IFS to illuminate five major challenges created by stepfamily structure. For each challenge, three levels of clinical work help extreme parts relax and support the Self-leadership required to meet these challenges: I. *Psychoeducational*: Providing information about how stepfamilies are different from first-time families, what works, and what doesn't to meet the challenges. II. *Interpersonal*: Supporting Self-to-Self connection in the face of the divisive forces of stepfamily structure. III. *Intrapsychic*: When reactivity remains high (or low), using IFS to heal old family-of-origin bruises that may be driving reactivity (Papernow, 2013).

Patricia Papernow: *Surviving and Thriving in Stepfamily Relationships: What Works and What Doesn't*, www.stepfamilyrelationships.com, ppapernow@gmail.com

Children and Adolescents

In her adaptation of IFS for children and adolescents, Pamela Krause points out that kids, who are powerless relative to adults, must influence family dynamics and adult behavior indirectly. Symptoms, which symbolize that indirection, lead us to the parts who problem-solve internally.

Krause illustrates how we can use IFS to help parents unblend from their protectors while children externalize and get into relationship with their parts, so they can validate the concerns of protectors and heal their exiled wounds. (Krause, 2013).

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Parenting Adult Children

In his adaptation of IFS to the challenges of parenting adult children, Paul Neustadt differentiates Self-led parenting from reactive parenting. The Self-led parent can see the present with perspective and clarity while the parent who is blended with protectors reacts to the present situation as if it is a negative experience from the past. Neustadt illustrates how the IFS approach can help parents unblend from protectors and parent from the Self (Neustadt, 2016).

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Groups

Group therapy is an umbrella term that covers a wide variety of options for a variety of very different populations. Although as yet there are no publications that describe how to use IFS in group therapy, many skilled practitioners are using IFS with groups. Our observation is that the basic principles of IFS (everyone has parts and when protective parts differentiate the Self is there to heal injured parts) can be adapted to group therapy for many client populations, including trauma survivors, depression, anxiety, eating disorders and addiction.

Movement

In her somatically-based adaptation of IFS, Susan McConnell locates the physical manifestations of attachment injuries and trauma and illustrates how the IFS therapist can use breath, movement and touch to embody the Self, be in relationship with parts and from this vantage witness, unburden and integrate these parts back into the internal system (McConnell, 2013).

Susan McConnell: susanmcon@gmail.com, embodiedself.net

TRAUMA

Post-Traumatic Stress Disorder (PTSD), Dissociative Identity Disorder (DID), Disorders of Extreme Stress Not Otherwise Specified (DESNOS)

In their application of IFS with trauma, Frank Anderson and Martha Sweezy differentiate IFS from standard trauma treatment: Standard treatment divides therapy into phases and offers various practices that are intended to help the client stabilize before moving on to some variety of exposure-based options for processing traumatic memories. In contrast, IFS welcomes extreme symptoms or parts at the onset, developing (inside and out) a way of relating that culminates in love and has the relational power to disconfirm the various assaultive falsehoods that trauma promotes (*I am unlovable, I am worthless*) (Anderson & Sweezy, 2016).

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Martha Sweezy, Northampton, MA, 617-669-7656, <http://marthasweezy.com>

The Trauma of Chronic Physical illness and Physical Illness Related to Trauma

In her adaptation of IFS to chronic illness, Nancy Sowell illustrates the ways in which soothing and healing the heart with the practice of profound self-acceptance in IFS eases the emotional and physiological dysregulation that typically follows trauma. Additionally, the conflicts that often accompany chronic illness, such as stoicism vs. fear and sadness, soften. This softening leads to improved self-compassion, empowerment and healing for body as well as heart (Sowell, 2013).

Nancy Sowell: www.nancysowell.com

MENTAL ILLNESSES THAT ARE TRAUMATIC

Bipolar Disorder and Schizophrenia

No study has been done to date and we don't know of any publications on the topic of IFS being used to treat major mental illness. We do know from clinical experience that major mental illness is in itself traumatic and we look forward to exploring the use of IFS with clients who are having these experiences.

MENTAL ILLNESSES THAT MAY RELATE TO TRAUMA AS WELL AS BIOLOGY

Depression and Anxiety

In the process of exploring the particular strengths of IFS, Martha Sweezy speculates that IFS is effective with a broad swath of psychic suffering because self-compassion, which is the

centerpiece of IFS, is mutually exclusive with self-shaming, and shame has a seminal role in a wide variety of symptoms, including depression and anxiety (Sweezy, 2016).

Martha Sweezy, Northampton, MA, 617-669-7656, <http://marthasweezy.com>

SEXUALITY

Welcoming All Sexual Parts and Reactions to the Erotic

Employing the concepts of IFS in relation to sexuality, Larry Rosenberg points out that parts internalize polarized cultural messages about sexuality – ranging from excited desire to shameful inhibition – and he posits that the erotic consists of essential yin-yang polarizations, with erotic stimulation benefiting from the opportunity to play with conflicting tensions. When therapists unburden their anxious or judging parts they can help clients explore their sexual functioning, identities, desires and behaviors (Rosenberg, 2013).

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LOSS

Self-led Grieving

In his adaptation of IFS to grief therapy, Derek Scott differentiates between simple and complicated grief. The former is straightforward and requires the IFS therapist to embody Self-energy while being a companion and guide (as needed) for the bereaved client. The latter, complicated grief, leads to parts who have experienced unsupported loss in the past and who need the love and compassion of the client's Self in order to heal (Scott, 2016).

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OPPRESSION

Racism (and all forms of othering or bigotry, including homophobia, transphobia, misogyny, xenophobia)

To address his own racism, Richard Schwartz befriended his racist parts and learned about their protective roles. He points out that the challenge of acknowledging our bigotry, especially our hidden bigotry, is less of a minefield when we think and speak in terms of parts – and the Self –

because the racist beliefs and behavior of some of our parts does not define us globally. In addition, he illustrates how helping all our parts feel securely attached and safe internally frees aggressive protectors from their onerous (mostly disowned) jobs. By facilitating Self-led conversations, Schwartz has been able to mobilize people – even people who are at war (specifically, the Israelis and the Palestinians) – to address the injuries of racism at both a personal and a communal level (Schwartz, 2016).

Richard Schwartz: <http://www.selfleadership.org>

EXTREME PROTECTION

Perpetrator Parts

Using IFS in therapy with perpetrators, Richard Schwartz learned that perpetrator parts are a distinct class of protector: they have a drive to dominate and humiliate, they experience relief when they are in a position of power, they despise and make a point of punishing vulnerability internally and in others, and they show no concern for consequences or the feelings of those whom they victimize. Although these parts are destructive and scary, Schwartz points out that they were not born to behave this way or do this job. In fact, protectors don't like doing their jobs and they do change once the parts they protect are healed (Schwartz, 2016).

Richard Schwartz: www.selfleadership.org

THE COSTLY DISTRACTION OF ADDICTIONS

Alcohol and Drugs

In her adaptation of IFS to addiction treatment, Cece Sykes helps clients see the typical addict polarity between harsh self-management and compulsive risk-taking and how protectors caught in both roles have positive intentions. Her overarching goal is help clients achieve the self-compassion they need to restore inner balance and heal (Sykes, 2016).

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Pornography

Avoiding the standard shaming approach of treatment for sex addicts, Nancy Wonder's first aim as she applies IFS with pornography addicts is for the therapist to be Self-led while exploring the inner polarity that grips the client: a part who loves porn and another part who views the

client as weak, embarrassing and despicable for needing porn to distract and self-soothe. Welcoming the protectors on both sides of this polarity helps the client become Self-led, heal exiled parts and let go of the compulsive illusion of comfort (Wonder, 2013).

Nancy Wonder specializes in attachment wounding, sexual abuse, and sexual acting-out: 850-222-7112, nancywonder@icloud.com

Eating Disorders

Anorexia, Bulimia, Binge-eating

Describing the use of IFS with eating disorders, Jeanne Catanzaro explains that the ED client has polarized protectors who are afraid of each other and of the exiled feelings that arise when they relax. Using IFS, the therapist can validate the fears of these well-intended parts and deescalate their conflict in order to access and heal the underlying injury (Catanzaro, 2016).

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BEYOND PSYCHOTHERAPY

Psychopharmacology

In his adaptation of IFS to psychopharmacology, Frank Anderson articulates five strategies for prescribing. First he identifies a list of symptoms and opens a dialogue with parts so he can differentiate a biological condition from the behavior of a part. Next he validates parts' previous experiences with medication and addresses their concerns about the future. After this, he prescribes only if all parts are in agreement. Once consensus is reached, he educates parts about what to expect from medications and invites them to stay in communication about their experience. Last, he helps his own parts to unblend so that he can be Self-led in the role of educator while the client makes the decisions (Anderson, 2013).

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Mindfulness

In his introduction to *Internal Family Systems Therapy: New Dimensions* Jack Engler writes about his personal therapy experiences with IFS and some notable parallels and differences between Schwartz's concept of "Self" and the teachings of various spiritual traditions. While most spiritual practices seek to access that which is already whole, aware and awake in us, Engler notes that there are a couple of key differences between the IFS approach and most

spiritual traditions: first, the Self is interactive and, second, accessing the Self does not require years of disciplined practice (Engler, 2013).

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Health coaching

In their adaptation of IFS to health coaching (Self-aware informational nonjudgmental health coaching or SINHC™), John Livingstone and Joanne Gaffney teach health coaches to be aware of the feelings and beliefs of their own parts so they can be emotionally available to the patient. Using direct access with the patient's parts (called "information interweave™") SINHC trained coaches do not offer advice at the outset of contact. Instead, they listen for the feelings and beliefs of the patient's parts. By expressing interest and validating concerns, the coach helps protective parts relax so the patient can hear evidence-based information and make Self-led decisions. (Livingstone & Gaffney, 2013) Joanne Gaffney: 508 487-0400, jgaffneyliving@gmail.com

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Creativity

In her introduction to *Innovations and Elaborations in Internal Family Systems Therapy*, Janna Malamud Smith compares the IFS psychotherapy process, in which psychic processes are concretized as clients and parts interact, with the way fiction, poetry and drama tap into the same essential process of mind as writers locate and inhabit different perspectives – not only of their characters but of their characters' parts (Smith, 2016).

Janna Smith is a regular contributor to WBUR's Cognoscenti: Jannamalamudsmith.com, jannamsmith@verizon.net

Research Related to IFS

Michael Mithoefer and his research group have been studying the effects of the drug 3,4-methylenedioxymethamphetamine (MDMA – recreational name Ecstasy) to assist in psychotherapy with war veterans, firefighters, police officers, and individuals who have PTSD due to rape, assault or childhood abuse. Mithoefer, a psychiatrist who is trained in IFS, wrote:

“MDMA fosters a pronounced increase in Self energy in clients who take the drug, along with an increased awareness of their parts and the ability to differentiate from them.” His results, both short and long term, have been remarkable for the reduction of PTSD symptoms.

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A study published in the *Journal of Rheumatology* in 2013 (Shadick, et al) showed that IFS treatment has favorable effects on patients with rheumatoid arthritis. This study was submitted to NREPP (the National Registry for Evidence-based Programs and Practices), which resulted in their endorsement of IFS as an *evidence-based practice*. In particular, they found promising effects of IFS on the mind (depression, anxiety), the body (physical health conditions), and the spirit (personal resilience and self-concept).

Recently, the Foundation for Self Leadership, a non-profit organization dedicated to the advancement of IFS through research, scholarship and advocacy beyond psychotherapy, has supported two pilot studies entitled: *IFS Therapy for the Treatment of PTSD and Complex Trauma* and *Exploring the Phenomenology, Physiology, and Dyadic Processes of an IFS Intervention*. The latter study hypothesizes that IFS has physiological effects on the therapist, the client, and the therapeutic relationship between them. Meanwhile, results from the PTSD and Complex Trauma study demonstrate significant reductions in symptoms of PTSD and depression in 12 of the 13 subjects after completing 16 weeks of IFS treatment. If you are interested in learning more about IFS research please visit the Foundation for Self Leadership online. FoundationIFS.org